

IMAGES IN CLINICAL MEDICINE

Cutaneous Leishmaniasis

The Images in Clinical Medicine featured in this issue of the Journal spotlight some of the effects of the climate crisis on clinical health.



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A PREVIOUSLY HEALTHY 3-YEAR-OLD BOY WAS BROUGHT TO THE EMERGENCY DEPARTMENT WITH A 6-WEEK HISTORY of painless bumps and ulcers on his left leg. He initially had itchy insect bites on his foot, and then bumps and ulcers had developed slowly on his leg. The boy had recently immigrated from Venezuela after a months-long journey by land with his family. On physical examination, ulcerated plaques with rolled borders and satellite papules were observed on the left lower leg, buttocks, and back. Three biopsy samples were obtained from one of the lesions. Histopathological analysis was notable for severe lymphohistiocytic infiltration with no organisms identified by routine microbiologic stains. Polymerase-chain-reaction testing of a tissue sample identified *Leishmania guyanensis*. A diagnosis of cutaneous leishmaniasis was made. Leishmaniasis is caused by a protozoa parasite that is transmitted by phlebotomine sandfly bites. Cutaneous leishmaniasis is the most common manifestation of the disease and may result in disfiguring scars. Climate change is affecting the spread of leishmaniasis by altering the geographic distribution and size of sandfly populations. People experiencing poverty and migration are at highest risk for acquiring the infection. After a 28-day course of miltefosine, the skin lesions resolved.

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